

個案報告中英文摘要表

計畫名稱：(中文)

骨盆底巨大硬皮囊腫擬似直腸惡性腫瘤：病例報告

計畫名稱：(英文)

A Large Epidermoid Cyst of the Pelvic Floor Mimicking Rectal Malignancy: A Case Report

試驗目的：

報告一例骨盆底巨大硬皮囊腫在 CT 上擬似直腸惡性腫瘤之罕見案例，描述術前診斷重新定向之關鍵特徵及手術處置策略

研究設計：

單一機構回溯性病例報告 (Retrospective single-case report)

試驗計劃摘要 (中文)：

骨盆底硬皮囊腫 (epidermoid cyst) 為極罕見良性病灶 (文獻報告少於 20 例)，電腦斷層掃描 (CT) 上易與惡性腫瘤混淆，造成診斷困難。一名 61 歲男性因下腹疼痛、血尿、排便習慣改變及裡急後重就醫。顯影電腦斷層 (CT) 顯示左側低位直腸旁 10 公分腫塊，併見疑似轉移性淋巴結，初步鑑別診斷包括胃腸道基質瘤 (GIST)、淋巴瘤及神經內分泌腫瘤。經三項術前處置發現協助導向正確診斷：大腸鏡無黏膜病灶、腫瘤標記 (CEA、AFP、CA19-9) 均在正常範圍，以及 CT 腫塊未見顯影。泌尿科會診 CT 判讀及術中確認左側中重度腎水腫，術前預防性置入雙 J 管。患者接受剖腹手術完整切除骨盆底腫塊，病理報告確認為硬皮囊腫，術後恢復良好，第 9 天出院。骨盆底硬皮囊腫應列入骨盆腔腫塊之鑑別診斷，完整手術切除為確定性治療，預後良好。

關鍵字：硬皮囊腫；骨盆底；骨盆腫塊；鑑別診斷；剖腹手術

試驗計劃摘要 (英文)：

Epidermoid cysts of the pelvic floor are rare benign lesions that can closely mimic malignancy on CT, posing significant diagnostic challenges. A 61-year-old male presented with lower abdominal pain, hematuria, altered bowel habits, and rectal tenesmus. Contrast-enhanced CT revealed a 10 cm pelvic mass with apparent lymphadenopathy, raising suspicion for GIST, lymphoma, or neuroendocrine tumor. Three pre-operative findings redirected the diagnosis: negative colonoscopy, normal tumor markers (CEA, AFP, CA19-9), and absent mass enhancement on CT. Urology consultation identified moderate-to-severe left hydronephrosis on CT review, prompting prophylactic double-J ureteral stenting. Laparotomy with

complete excision confirmed an epidermoid cyst on histopathology. The patient was discharged on postoperative day 9 without complications. When a non-enhancing extrinsic pelvic mass is accompanied by negative colonoscopy and normal tumor markers, epidermoid cyst should be included in the differential. Complete surgical excision offers an excellent prognosis.

Key Word : epidermoid cyst; pelvic floor; pelvic mass; differential diagnosis; laparotomy